NMED | WHAT BRINGS A CUSTOMER BACK, AND HOW WE DEFINE RETENTION

President AI Award 2026, M2 Validation, Task 2.3. Sources: customer interview of 30 Oct 2026 with Rahmonov Rahmatullo (Head of Cardiology, NEOMED Hospital; Chief Physician, Cardio Service Clinic); NMED production data and clinic staffing as of 31 Oct 2026. Estimates are marked as such.

01 THE RECURRING JOB

Our customer is a clinic. The chief physician or head of department evaluates NMED, and the director approves payment based on that assessment. Daily users are nurses, who upload test files, and physicians, who review and sign off the reports. The recurring job: every working day, interpret the incoming ECG, Holter, ambulatory blood pressure monitoring (ABPM) and laboratory results, write a report and keep it in the patient's history. The job does not end as long as the clinic runs tests: both clinics work 6 days a week.

Current customers. NEOMED Hospital: free pilot in the cardiology department (3 physicians, 5 nurses). Cardio Service Clinic: whole clinic, one-month free trial started on 20 Oct 2026 (11 physicians, 15 nurses). Both clinics stated they are willing to sign a letter of intent for a monthly subscription after the trial.

02 WHAT TRIGGERS THE NEXT USE

- **A new test.** A patient has an ECG, Holter, ABPM or lab test → a nurse uploads the file → NMED's AI drafts a preliminary report → the test is assigned to a physician and appears as "unviewed" in their list. This is the main, daily trigger.
- **A patient comes back.** At a follow-up visit, staff find the patient by passport and open the patient card with all previous results. With 2 or more results, NMED produces a combined AI conclusion that reviews 2–10 tests together.
- **A document is needed.** A PDF report for the patient, or a QR-verifiable discharge summary when an inpatient is discharged.

03 HOW OFTEN

Per clinic: every working day, dozens of tests (6-day week, about 26 working days a month). Monthly test volumes from the interview:

Test type	NEOMED (cardiology dept), per month	Cardio Service (whole clinic), per month	Physician time per test
ECG	≈ 700	≈ 2,500	15 min
Holter	≈ 30	≈ 50	1–1.5 h
ABPM	≈ 40	≈ 70	10–20 min
Laboratory	≈ 800	≈ 2,000	≈ 10 min
Total	**1,570** (≈ 60 per working day)	**4,620** (≈ 178 per working day)	≈ 345–367 h / ≈ 1,020–1,057 h per month

Per patient (physician's estimate for patients under follow-up): ECG every month, laboratory tests every month, ABPM every 2 months, Holter every 2 months. A chronic cardiac patient under follow-up therefore returns about 12 times a year, and every visit adds to the history that makes the next comparison and combined review more useful.

Actual NMED usage (production): September 2026: 1,220 analyses, NEOMED only (≈ 47 per working day, about 78% of the department's 1,570 monthly tests). October 2026: 3,936 analyses, NEOMED plus Cardio Service from 20 October.

Volumes, times and intervals are the interviewed physician's estimates, not independent measurements. Production counts come from the NMED database.

04 THE VALUE THEY COME BACK FOR

- **Physician time.** Interpreting tests currently takes ≈ 345–367 physician-hours a month at NEOMED and ≈ 1,020–1,057 at Cardio Service, equal to about 2 and 6 full-time physicians (170 h/month). NMED prepares the draft; the physician checks and confirms it.
- **Physician's assessment.** In the interview, NMED was described as reducing physician workload, improving data accuracy and saving time in diagnosis.
- **One patient history.** All results sit in one patient card: paper results are no longer lost, and comparison with earlier results is easy. The more history a clinic accumulates, the more useful the combined conclusion becomes, which also keeps the clinic on NMED.
- **Price.** Selected packages: NEOMED 1,500 analyses for UZS 9 million, Cardio Service 4,000 analyses for UZS 12 million (new plan), which is UZS 6,000 and UZS 3,000 per analysis.

05 OUR DEFINITION OF A RETAINED CUSTOMER

A retained customer is a clinic that uploads tests to NMED on at least 4 of its 6 working days every week and uses at least 50% of its subscription package every month (NEOMED ≥ 750, Cardio Service ≥ 2,000 analyses per month).

Why these thresholds. The subscription is paid per package. Below half of the package, it becomes expensive for the clinic and churn risk appears. "4 of 6 working days" shows that NMED is part of the daily workflow, not a batch upload once a week. Above 100% of the package, the clinic moves to a higher tier, as agreed in the interview. NEOMED's September usage (1,220 analyses, 81% of its package) already falls in the "retained" range. The thresholds are a hypothesis and will be refined after the first full month.

Package used per month	Meaning
Below 50%	Churn risk: the package looks expensive; the clinic downgrades or stops
50–100%	Retained customer
Above 100%	Expansion: upgrade to a higher package

06 HOW WE WILL TEST IT

- **Data.** For every analysis, NMED stores the clinic, the uploading staff member, the patient and the time, and records when a physician opens it. Metrics come from a weekly SQL report on the database, not from a survey.
- **Period.** 20 Oct to 20 Dec 2026: Cardio Service's free trial month plus its first paid month; NEOMED's cardiology department in parallel.
- **Weekly metrics per clinic.** Analyses against the package (pro rata), active working days, active staff (of 8 and 26), share of analyses opened by physicians, share of returning patients, and the actual return interval compared with the physician's 1–2-month estimate.
- **Pass criterion.** The hypothesis holds if both clinics meet the definition in the trial month and in the first paid month, sign the letter of intent and make the first payment. In any week below 50% of the package (pro rata), we contact the clinic and record the reason.
- **Limitations.** Volumes, times and intervals come from one physician, who evaluates NMED for both clinics. NEOMED is the founder's workplace and a free pilot, so it may be biased. No clinic has paid yet, so retention under paid conditions is not yet confirmed.